16.7 Infant of a Diabetic Mother*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective *No specific complaint* **Objective** • RR 72/min • Mild grunting and recession • 37 weeks, 4.3 kg	**ACTUAL** Ineffective breathing pattern related to delayed pulmonary maturity as evidenced by tachypnea, grunting and retractions.	• RR 40–60/min and SpO_2 on target within 24 hours.	• Follow RR, SpO_2 and work of breathing. • Give oxygen or CPAP as prescribed. • Keep a neutral thermal environment. • Report increasing distress.	• Goal reached when breathing is normal.
Subjective • Mother says, “The baby is sleepy and does not feed well.” **Objective** • Jittery • Sleepy at feeds • Large for gestational age	**ACTUAL** Ineffective infant feeding dynamics related to lethargy and jitteriness as evidenced by poor feeding.	• Feeds at least every 3 hours with enough intake.	• Wake the baby for feeds every 2–3 hours. • Support breastfeeding; give expressed milk if needed. • Record intake and output.	• Met when feeding well.
Subjective *No specific complaint* **Objective** • Maternal diabetes • Glucose 35 mg/dL at 2 hours of age	**RISK** Risk for physical injury as evidenced by risk factors: neonatal hypoglycemia associated with maternal diabetes and neonatal hyperinsulinism.	• Blood glucose normal for age.	• Feed within 1 hour of birth and often after that. • Check glucose before feeds per protocol. • Give IV dextrose as prescribed if glucose stays low. • Watch for jitteriness, poor feeding and seizures.	• Achieved while glucose is normal.

*References for this care plan: [3], [35], [38]. Full entries are listed in the References section of the CPMN book.